Tracer Observation Form

Comprehensive Documentation AW - CURRENT

Site: 486045 Midwest Orthopedic Specialty Hospital

Program: Hospital

Observation Header	
Tracer Instructions: *No Instructions Provided*	
Observation Title:	
Observation Date:	
Total Completed Observations:	**Department:**
Survey Team:	**Staff Interviewed:**
Medical Staff:	**Unique Identifier:**
Location:	**Equipment Observed:**
	Contracted Service:
Note:	

Questions (* = Required)

Advanced Directives

1. Advanced directives are addressed, if no advance directive is present was the patient offered assistance.
(Not applicable in the ED) ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

RC.12.01.01 EP 3 - The medical record contains any informed consent, when required by hospital policy or federal or state law or regulation. Note: The properly executed informed consent is placed in the patient's medical record prior to surgery, except in emergencies. A properly executed informed consent contains documentation of a patient's mutual understanding of and agreement for care, treatment, and services through written signature; electronic signature; or, when a patient is unable to provide a signature, documentation of the verbal agreement by the patient or surrogate decision-maker.

Corrective Action:

Notes:

H&P

2. Is a comprehensive History and Physical (H&P) in the EHR? ☐ N/A

Instructions: Notes: If a H&P is completed within 30 days of admission, the physician will need to perform a follow-up assessment and document it within 24 hours after patient registration or prior to surgery/invasive procedure.
Notes: Required elements include 1) statement of complaint, 2) health history, 3) statement about the results of the physical examination, and 4) provisional diagnosis.

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.02.01 EP 2 - A medical history and physical examination is completed and documented no more than 30 days prior to, or within 24 hours after, registration or inpatient admission but prior to surgery or a procedure requiring anesthesia services. Note 1: For hospitals that use Joint Commission accreditation for deemed status purposes: Medical histories and physical examinations are performed as required in this element of performance, except prior to any specific outpatient surgical or procedural services for which an assessment is performed instead as provided under 42 CFR 482.24(c)(4)(i)(C). Note 2: For law and regulation guidance pertaining to the medical history and physical examination at 42 CFR 482.22(c)(5)(iii) and 482.51 (b)(1)(iii), refer to https://www.ecfr.gov/.

Corrective Action:

Notes:

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Pain Assessment/Reassessment

3. If the patient's admission pain screening is positive, is a pain management goal documented? ☐ N/A

Instructions: Pain goal can be patient-reported pain score or therapeutic activity goal.

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.06.02.01 EP 1 - The hospital has defined criteria to screen, assess, and reassess pain that are consistent with the patient's age, condition, and ability to understand.

Corrective Action:

Notes:

4. Was the patient assessed for pain at a minimum of two times in 24 hours? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.02.01 EP 9 - The hospital defines, in writing, the scope and content of screening, assessment, and reassessment. Patient information is collected according to these requirements. Note 1: In defining the scope and content of the information it collects, the hospital may want to consider information that it can obtain, with the patient's consent, from the patient's family and the patient's other care providers, as well as information conveyed on any medical jewelry. Note 2: Assessment and reassessment information includes the patient's perception of the effectiveness of, and any side effects related to, their medication(s).

Corrective Action:

Notes:

5. Was the sedation level/LOC assessed prior to opioid administration for patients receiving opioids to manage pain? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.06.01.01 EP 1 - The hospital has a leader or leadership team that is responsible for pain management and safe opioid prescribing, as well as developing and monitoring performance improvement activities.

Corrective Action:

Notes:

6. Was the sedation level/LOC assessed after opioid administration for patients receiving opioids to manage pain? ☐ N/A

Instructions: All hospitals reassess after IV opioid 60 minutes, PO opioid 120 minutes.

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.02.01 EP 9 - The hospital defines, in writing, the scope and content of screening, assessment, and reassessment. Patient information is collected according to these requirements. Note 1: In defining the scope and content of the information it collects, the hospital may want to consider information that it can obtain, with the patient's consent, from the patient's family and the patient's other care providers, as well as information conveyed on any medical jewelry. Note 2: Assessment and reassessment information includes the patient's perception of the effectiveness of, and any side effects related to, their medication(s).

Corrective Action:

Notes:

7. Was the correct pain medication and dose given based on the PRN indication and the patient's reported pain level? ☐ N/A

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Instructions: When a pain med with PRN indication includes pain intensity, compare the patient's reported pain score documented at the time med was given to the PRN indication for the med given. Does the patient's pain score match the PRN indication for med given?
YES response if patient preference for lower dose, less potent pain med is documented.
NO response if no pain score is documented

Mild pain = 1 - 3/10 (Numeric, FPS-R scales)
Moderate pain = 4 - 6/10 (Numeric, FPS-R scales)
Severe pain = 7 - 10/10 (Numeric, FPS-R scales)

Compliant? ◯ Yes ◯ No	Num: _____ Den: _____

NPG.06.02.01 EP 3 - The hospital treats the patient's pain or refers the patient for treatment. Note: Treatment strategies for pain may include nonpharmacologic, pharmacologic, or a combination of approaches.

Corrective Action:

Notes:

8. Review the medical record for the most recent 24 hours. Is there a pain reassessment after each intervention within the allotted time frame according to policy? ☐ N/A

Instructions: Parenteral route - within 1 hour
Enteral route - within 2 hours
Nonpharmacologic - within 4 hours

Compliant? ◯ Yes ◯ No	Num: _____ Den: _____

NPG.06.02.01 EP 2 - The hospital screens patients for pain during emergency department visits and at the time of admission.

Corrective Action:

Notes:

Pressure Injury Prevention

9. Is the initial head to toe skin assessment completed (4 eyes) documented upon admission? (Applicable to ALL inpatient departments) ☐ N/A

Compliant? ◯ Yes ◯ No	Num: _____ Den: _____

PC.11.02.01 EP 9 - The hospital defines, in writing, the scope and content of screening, assessment, and reassessment. Patient information is collected according to these requirements. Note 1: In defining the scope and content of the information it collects, the hospital may want to consider information that it can obtain, with the patient's consent, from the patient's family and the patient's other care providers, as well as information conveyed on any medical jewelry. Note 2: Assessment and reassessment information includes the patient's perception of the effectiveness of, and any side effects related to, their medication(s).

Corrective Action:

Notes:

10. Is Braden score assessed upon admission and at least every shift thereafter? ☐ N/A

Compliant? ◯ Yes ◯ No	Num: _____ Den: _____

PC.11.02.01 EP 9 - The hospital defines, in writing, the scope and content of screening, assessment, and reassessment. Patient information is collected according to these requirements. Note 1: In defining the scope and content of the information it collects, the hospital may want to consider information that it can obtain, with the patient's consent, from the patient's family and the patient's other care providers, as well as information conveyed on any medical jewelry. Note 2: Assessment and reassessment information includes the patient's perception of the effectiveness of, and any side effects related to, their medication(s).

Corrective Action:

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Notes:

11. Are every 2hr turns (or as ordered) and documented appropriately over the past 24 hours? NOTE: For a Braden score of 18 or below, q2hour turns are required. Documentation of turns is NOT required if the Braden score is 19 or above. ☐ N/A

Instructions: For a Braden score of 18 or below, q2hour turns are required. Documentation of turns is NOT required if the Braden score is 19 or above.

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NR.11.01.01 EP 2 - All licensed nurses who provide services in the hospital adhere to its policies and procedures. Note: This applies to all nursing staff providing services (that is, hospital employee, contract, lease, other agreement, or volunteer).

Corrective Action:

Notes:

12. Were all relevant Braden scale risk directed interventions (scores of 1 or 2) completed every shift during the most recent 24 hours? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.02.01 EP 9 - The hospital defines, in writing, the scope and content of screening, assessment, and reassessment. Patient information is collected according to these requirements. Note 1: In defining the scope and content of the information it collects, the hospital may want to consider information that it can obtain, with the patient's consent, from the patient's family and the patient's other care providers, as well as information conveyed on any medical jewelry. Note 2: Assessment and reassessment information includes the patient's perception of the effectiveness of, and any side effects related to, their medication(s).

Corrective Action:

Notes:

13. Are heels offloaded every 2hrs (or as ordered) and documented appropriately over the past 24 hours? NOTE: For a Braden score of 18 or below, q2hour turns are required. Documentation of turns is NOT required if the Braden score is 19 or above. ☐ N/A

Instructions: For a Braden score of 18 or below, q2hour turns are required. Documentation of turns is NOT required if the Braden score is 19 or above.

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NR.11.01.01 EP 2 - All licensed nurses who provide services in the hospital adhere to its policies and procedures. Note: This applies to all nursing staff providing services (that is, hospital employee, contract, lease, other agreement, or volunteer).

Corrective Action:

Notes:

14. If applicable, did the nurse complete a wound assessment upon discovery, and with dressing changes? ☐ N/A

Instructions: Mark N/A if the patient does not have wounds.

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.02.01 EP 9 - The hospital defines, in writing, the scope and content of screening, assessment, and reassessment. Patient information is collected according to these requirements. Note 1: In defining the scope and content of the information it collects, the hospital may want to consider information that it can obtain, with the patient's consent, from the patient's family and the patient's other care providers, as well as information conveyed on any medical jewelry. Note 2: Assessment and reassessment information includes the patient's perception of the effectiveness of, and any side effects related to, their medication(s).

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Corrective Action:

Notes:

15. If applicable, did the nurse initiate wound interventions (cleansing, dressings) upon discovery and per MD/WOCN orders? ☐ N/A

Instructions: Mark N/A if the patient does not have wounds.

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.02.01 EP 9 - The hospital defines, in writing, the scope and content of screening, assessment, and reassessment. Patient information is collected according to these requirements. Note 1: In defining the scope and content of the information it collects, the hospital may want to consider information that it can obtain, with the patient's consent, from the patient's family and the patient's other care providers, as well as information conveyed on any medical jewelry. Note 2: Assessment and reassessment information includes the patient's perception of the effectiveness of, and any side effects related to, their medication(s).

Corrective Action:

Notes:

Restraint

16. At the time of initiation of restraints, is a restraint application assessment and the restraint intervention used, completed and documented immediately after application using the restraint flowsheet in EPIC? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.02.09 EP 1 - The hospital's policies and procedures regarding the use of restraint or seclusion include the following: - Definitions for restraint and seclusion that are consistent with state and federal law and regulation - Physician and other licensed practitioner training requirements - Staff training requirements - Who has authority to order restraint or seclusion - Who has authority to discontinue the use of restraint or seclusion - Who can initiate the use of restraint or seclusion - Circumstances under which restraint or seclusion is discontinued - Requirement that restraint or seclusion is discontinued as soon as is safely possible - Who can assess and monitor patients in restraint or seclusion - Time frames for assessing and monitoring patients in restraint or seclusion

Corrective Action:

Notes:

17. Does documentation clearly indicate the restraint type ordered was appropriate to protect the immediate physical safety of the patient, staff, or others? Is the documentation in accordance with hospital policy? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.02.15 EP 1 - Documentation of restraint or seclusion in the medical record includes the following: - The 1-hour face-to-face medical and behavioral evaluation if restraint or seclusion is used to manage violent or self-destructive behavior - Description of the patient's behavior and the intervention used - Alternatives or other less restrictive interventions attempted (as applicable) - Patient's condition or symptom(s) that warranted the use of the restraint or seclusion - Patient's response to the intervention(s) used, including the rationale for continued use of the intervention

Corrective Action:

Notes:

18. At the time of initiation of restraints, does the documentation indicate that alternatives or less restrictive methods were ineffective prior to utilizing restraints? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.02.01 EP 1 - The hospital does not use restraint or seclusion of any form as a means of coercion, discipline, convenience, or staff retaliation. Restraint or seclusion is only used to protect the immediate physical safety of the patient, staff, or others when less restrictive interventions have been ineffective and is discontinued at the earliest possible time, regardless of the length of time specified in the order.

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Corrective Action:

Notes:

19. Was the restraint order renewed at the frequency defined within the policy for type of restraint and patient population? ☐ N/A

Numerator = number of renewal orders written or entered in accordance with policy
Denominator = total number of renewal orders required

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.02.05 EP 6 - Orders for restraint used to protect the physical safety of a nonviolent or non-self-destructive patient are renewed in accordance with hospital policy.

Corrective Action:

Notes:

20. Does the type of restraint applied match the type of restraint ordered, both the initial, and any renewal orders? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.02.05 EP 1 - The hospital uses restraint or seclusion as ordered by a physician or other authorized licensed practitioner responsible for the patient's care in accordance with hospital policy and state law and regulation.

Corrective Action:

Notes:

21. Were all assessments and reassessments documented at the time frames defined by policy? ☐ N/A

Numerator = number of assessments and reassessments documented in accordance with policy
Denominator = total number of assessments and reassessments required

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.02.09 EP 1 - The hospital's policies and procedures regarding the use of restraint or seclusion include the following: - Definitions for restraint and seclusion that are consistent with state and federal law and regulation - Physician and other licensed practitioner training requirements - Staff training requirements - Who has authority to order restraint or seclusion - Who has authority to discontinue the use of restraint or seclusion - Who can initiate the use of restraint or seclusion - Circumstances under which restraint or seclusion is discontinued - Requirement that restraint or seclusion is discontinued as soon as is safely possible - Who can assess and monitor patients in restraint or seclusion - Time frames for assessing and monitoring patients in restraint or seclusion

Corrective Action:

Notes:

22. Is a face to face evaluation conducted and documented at minimum each calendar day the patient remains in restraints? ☐ N/A

Instructions: The evaluation must include the following: - An evaluation of the patient's immediate situation - The patient's reaction to the intervention - The patient's medical and behavioral condition - The need to continue or terminate the restraint or seclusion (N: Number of days in restraints with face to face documented D: Total number of days in restraints)

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.02.11 EP 1 - A physician or other licensed practitioner responsible for the patient's care evaluates the patient in person within one hour of the initiation of restraint or seclusion used for the management of violent or self-destructive behavior that jeopardizes the physical safety of the patient, staff, or others. A registered nurse may conduct the in-person evaluation within one hour of the initiation of restraint or seclusion if they are trained in accordance with the requirements in PC.13.02.17, EP 3. Note: The hospital also follows any state statute or regulation that may be more stringent than the requirements in this element of performance.

Corrective Action:

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Notes:

23. Is a face to face evaluation conducted within 24 hours of the initiation of restraints for the management of non-violent behavior via verbal order? ☐ N/A

Instructions: The evaluation must include the following: - An evaluation of the patient's immediate situation - The patient's reaction to the intervention - The patient's medical and behavioral condition - The need to continue or terminate the restraint or seclusion

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.02.11 EP 1 - A physician or other licensed practitioner responsible for the patient's care evaluates the patient in person within one hour of the initiation of restraint or seclusion used for the management of violent or self-destructive behavior that jeopardizes the physical safety of the patient, staff, or others. A registered nurse may conduct the in-person evaluation within one hour of the initiation of restraint or seclusion if they are trained in accordance with the requirements in PC.13.02.17, EP 3. Note: The hospital also follows any state statute or regulation that may be more stringent than the requirements in this element of performance.

Corrective Action:

Notes:

24. Is the face to face evaluation conducted within one hour of the initiation of restraint or seclusion for the management of violent/self destructive behavior? ☐ N/A

Instructions: The evaluation must include the following: - An evaluation of the patient's immediate situation - The patient's reaction to the intervention - The patient's medical and behavioral condition - The need to continue or terminate the restraint or seclusion

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.02.01 EP 2 - The hospital uses the least restrictive form of restraint or seclusion that will be effective to protect the patient, a staff member, or others from harm.

Corrective Action:

Notes:

25. Was there documentation of patient and/or family education about the use of restraints? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.12.02.01 EP 3 - Based on the patient's condition and assessed needs, the education and training provided to the patient by the hospital include any of the following: - An explanation of the plan for care, treatment, and services - Basic health practices and safety - Information on the safe and effective use of medications - Nutrition interventions (for example, supplements) and modified diets - Discussion of pain, the risk for pain, the importance of effective pain management, the pain assessment process, and methods for pain management - Information on oral health - Information on the safe and effective use of medical equipment or supplies provided by the hospital - Habilitation or rehabilitation techniques to help the patient reach maximum independence - Fall reduction strategies

Corrective Action:

Notes:

26. Was the discontinuation of restraints documented by the nurse, including the time of removal? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.02.09 EP 1 - The hospital's policies and procedures regarding the use of restraint or seclusion include the following: - Definitions for restraint and seclusion that are consistent with state and federal law and regulation - Physician and other licensed practitioner training requirements - Staff training requirements - Who has authority to order restraint or seclusion - Who has authority to discontinue the use of restraint or seclusion - Who can initiate the use of restraint or seclusion - Circumstances under which restraint or seclusion is discontinued - Requirement that restraint or seclusion is discontinued as soon as is safely possible - Who can assess and monitor patients in restraint or seclusion - Time frames for assessing and monitoring patients in restraint or seclusion

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Corrective Action:

Notes:

27. Is there a restraint care plan in place for the patient in restraints? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.03.01 EP 1 - The hospital develops, implements, and revises a written individualized plan of care based on the following: - Needs identified by the patient's assessment, reassessment, and results of diagnostic testing - The patient's goals and the time frames, settings, and services required to meet those goals Note 1: Nursing staff develops and keeps current a nursing plan of care, which may be a part of an interdisciplinary plan of care, for each patient. Note 2: The hospital evaluates the patient's progress and revises the plan of care based on the patient's progress. Note 3: For psychiatric hospitals that use Joint Commission accreditation for deemed status purposes: The patient's goals include both short- and long-term goals.

Corrective Action:

Notes:

28. Was the manager or house supervisor notified for the initiation of restraint or seclusion for the management of violent/self destructive behavior? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.02.09 EP 1 - The hospital's policies and procedures regarding the use of restraint or seclusion include the following: - Definitions for restraint and seclusion that are consistent with state and federal law and regulation - Physician and other licensed practitioner training requirements - Staff training requirements - Who has authority to order restraint or seclusion - Who has authority to discontinue the use of restraint or seclusion - Who can initiate the use of restraint or seclusion - Circumstances under which restraint or seclusion is discontinued - Requirement that restraint or seclusion is discontinued as soon as is safely possible - Who can assess and monitor patients in restraint or seclusion - Time frames for assessing and monitoring patients in restraint or seclusion

Corrective Action:

Notes:

Suicide Ideation

29. Pre-screening questions are documented in the patient chart.
1) Is this patient being evaluated or treated for a behavioral health condition or concern?
2) Has the patient displayed warning signs or made statements of wishing to die/self harm or have you received information indicating a safety concern from a caregiver/other professional? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.08.01.01 EP 3 - The hospital uses an evidence-based process to conduct a suicide assessment of patients who have screened positive for suicidal ideation. The assessment directly asks about suicidal ideation, plan, intent, suicidal or self-harm behaviors, risk factors, and protective factors. Note: EPs 2 and 3 can be satisfied through the use of a single process or instrument that simultaneously screens patients for suicidal ideation and assesses the severity of suicidal ideation.

Corrective Action:

Notes:

30. Individuals 12 years and older presenting with a behavioral health complaint or with behaviors that indicate a risk of hurting themselves are screened for suicidal ideation using the C-SSRS.
NOTE: Pediatric patients ages 5 -11 years of age who will be screened for suicide risk when they are presenting with behavioral health chief complaints, if the patient or parent raises concern, there is a reported history of suicidal ideation or behavior, or if the patient displays warning signs of suicide. ☐ N/A

Instructions: Flowsheets - Screenings - C-SSRS Initial

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

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NPG.08.01.01 EP 3 - The hospital uses an evidence-based process to conduct a suicide assessment of patients who have screened positive for suicidal ideation. The assessment directly asks about suicidal ideation, plan, intent, suicidal or self-harm behaviors, risk factors, and protective factors. Note: EPs 2 and 3 can be satisfied through the use of a single process or instrument that simultaneously screens patients for suicidal ideation and assesses the severity of suicidal ideation.

Corrective Action:

Notes:

31. Individuals screened high risk for suicidal ideation based on results of the C-SSRS or per clinical judgement, had a 1:1 Constant Observer initiated immediately. ☐ N/A

Instructions: Summary - Flowsheets - Suicide/Safety Check - Visual Checks

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.08.01.01 EP 5 - The hospital follows written policies and procedures addressing the care of patients identified as at risk for suicide. At a minimum, these should include the following: - Training and competence assessment of staff who care for patients at risk for suicide - Guidelines for reassessment - Monitoring patients who are at high risk for suicide

Corrective Action:

Notes:

32. Individuals who screened high risk for suicide, had safety checks documented minimally every 15 minutes, or per provider order in Behavioral Health settings, until the discontinuation of 1:1 observation order. ☐ N/A

Instructions: Flowsheets - Suicide Check/Interventions - Room Check - Pt. Activity

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.08.01.01 EP 5 - The hospital follows written policies and procedures addressing the care of patients identified as at risk for suicide. At a minimum, these should include the following: - Training and competence assessment of staff who care for patients at risk for suicide - Guidelines for reassessment - Monitoring patients who are at high risk for suicide

Corrective Action:

Notes:

33. Individuals who screened moderate risk for suicide, had safety checks documented minimally every 30 minutes, or per provider order in Behavioral Health settings ☐ N/A

Instructions: Flowsheets - Suicide Check/Interventions - Room Check - Pt. Activity

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.08.01.01 EP 5 - The hospital follows written policies and procedures addressing the care of patients identified as at risk for suicide. At a minimum, these should include the following: - Training and competence assessment of staff who care for patients at risk for suicide - Guidelines for reassessment - Monitoring patients who are at high risk for suicide

Corrective Action:

Notes:

34. Individuals who screened high risk or moderate risk for suicide, had the patient room evaluated for safety risks, using the Safer Room checklist. ☐ N/A

Instructions: Flowsheets - Cares/Safety - Suicide Safer Room Checklist

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

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NPG.08.01.01 EP 5 - The hospital follows written policies and procedures addressing the care of patients identified as at risk for suicide. At a minimum, these should include the following: - Training and competence assessment of staff who care for patients at risk for suicide - Guidelines for reassessment - Monitoring patients who are at high risk for suicide

Corrective Action:

Notes:

35. Individuals who screened high risk or moderate risk for suicide, were placed in a ligature-resistant garment (no strings) and a search and the securement of patient's personal belongings not deemed safe is conducted. ☐ N/A

Instructions: Flowsheets - Cares/Safety - Suicide Safer Room Checklist

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.08.01.01 EP 2 - The hospital screens all patients for suicidal ideation who are being evaluated or treated for behavioral health conditions as their primary reason for care using a validated screening tool. Note: Joint Commission requires screening for suicidal ideation using a validated tool for patients age 12 and above.

Corrective Action:

Notes:

36. Individuals who screened high risk for suicide, were provided the 988 Suicide & Crisis Lifeline information. ☐ N/A

Instructions: Chart Review - Media / On Base - After Visit Summary

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.08.01.01 EP 6 - The hospital follows written policies and procedures for counseling and follow-up care at discharge for patients identified as at risk for suicide.

Corrective Action:

Notes:

37. Individuals who screened positive (moderate or high) for suicidal ideation, have a suicide risk assessment conducted by a provider including all of the following assessment points 1) Thoughts, 2) Method, 3) Plan, 4) Behaviors, 5) Intent of Suicidal Ideations including frequency, duration, controlability, deterrents, and reasons.
Note: Must be documented during the visit period. ☐ N/A

Instructions: Provider Notes: SAFE-T template -or- All elements present in assessment or other note

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.08.01.01 EP 4 - The hospital documents patients' overall level of risk for suicide and the plan to mitigate the risk for suicide.

Corrective Action:

Notes:

38. Following the suicide risk assessment by the provider, a subsequent suicide precaution order matching the providers suicide risk assessment determination is documented. ☐ N/A

Instructions: Orders -compared too- Provider Notes: SAFE-T template -or- All elements present in assessment or other note

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

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NPG.08.01.01 EP 5 - The hospital follows written policies and procedures addressing the care of patients identified as at risk for suicide. At a minimum, these should include the following: - Training and competence assessment of staff who care for patients at risk for suicide - Guidelines for reassessment - Monitoring patients who are at high risk for suicide

Corrective Action:

Notes:

39. Individuals who screened positive (moderate or high) for suicidal ideation have an active nursing plan of care is documented that addresses patient safety. ☐ N/A

Instructions: Nursing Care Plans

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.03.01 EP 1 - The hospital develops, implements, and revises a written individualized plan of care based on the following: - Needs identified by the patient's assessment, reassessment, and results of diagnostic testing - The patient's goals and the time frames, settings, and services required to meet those goals Note 1: Nursing staff develops and keeps current a nursing plan of care, which may be a part of an interdisciplinary plan of care, for each patient. Note 2: The hospital evaluates the patient's progress and revises the plan of care based on the patient's progress. Note 3: For psychiatric hospitals that use Joint Commission accreditation for deemed status purposes: The patient's goals include both short- and long-term goals.

Corrective Action:

Notes:

40. Individuals who screened positive (moderate or high) for suicidal ideation have the C-SSRS re-screening completed during the first patient interaction/assessment of every shift in all departments/units. ☐ N/A

Instructions: Flowsheets - Screenings - Suicidal Ideation Rescreen

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.08.01.01 EP 3 - The hospital uses an evidence-based process to conduct a suicide assessment of patients who have screened positive for suicidal ideation. The assessment directly asks about suicidal ideation, plan, intent, suicidal or self-harm behaviors, risk factors, and protective factors. Note: EPs 2 and 3 can be satisfied through the use of a single process or instrument that simultaneously screens patients for suicidal ideation and assesses the severity of suicidal ideation.

Corrective Action:

Notes:

41. When suicide assessment SI risk level was increased or decreased, the provider order entered reflecting the appropriate interventions.
Numerator = Number of times the adjusted SI risk level had the correct level of intervention ordered after the change.
Denominator = Number of times the SI risk level was adjusted. ☐ N/A

Instructions: Orders -compared too- Provider Notes: SAFE-T template -or- All elements present in assessment or other note.

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.08.01.01 EP 5 - The hospital follows written policies and procedures addressing the care of patients identified as at risk for suicide. At a minimum, these should include the following: - Training and competence assessment of staff who care for patients at risk for suicide - Guidelines for reassessment - Monitoring patients who are at high risk for suicide

Corrective Action:

Notes:

Learning needs assessment

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42. Has the learning needs assessment been completed upon admission? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.12.02.01 EP 1 - The hospital performs a learning needs assessment for each patient, which includes the following: - Cultural and religious beliefs - Emotional barriers - Desire and motivation to learn - Physical or cognitive limitations - Barriers to communication

Corrective Action:

Notes:

Care plans

43. Do the nursing care plans reflect the patient's current condition/needs? ☐ N/A

Instructions: Examples:
Patients receiving pain medications have a care plan for acute pain
Patients receiving insulin have a care plan for Diabetes
Patients with risk for pressure injury or wounds have a care plan for skin integrity risk or patients with wounds have a care plan for skin impairment wounds
Care plans reflect any changes in the patient's hospital course (hospital acquired infections, blood administration, intubated into their stay, bowel obstructions, etc.)

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.12.02.01 EP 3 - Based on the patient's condition and assessed needs, the education and training provided to the patient by the hospital include any of the following: - An explanation of the plan for care, treatment, and services - Basic health practices and safety - Information on the safe and effective use of medications - Nutrition interventions (for example, supplements) and modified diets - Discussion of pain, the risk for pain, the importance of effective pain management, the pain assessment process, and methods for pain management - Information on oral health - Information on the safe and effective use of medical equipment or supplies provided by the hospital - Habilitation or rehabilitation techniques to help the patient reach maximum independence - Fall reduction strategies

Corrective Action:

Notes:

44. Is the care plan individualized and include target dates? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.03.01 EP 1 - The hospital develops, implements, and revises a written individualized plan of care based on the following: - Needs identified by the patient's assessment, reassessment, and results of diagnostic testing - The patient's goals and the time frames, settings, and services required to meet those goals Note 1: Nursing staff develops and keeps current a nursing plan of care, which may be a part of an interdisciplinary plan of care, for each patient. Note 2: The hospital evaluates the patient's progress and revises the plan of care based on the patient's progress. Note 3: For psychiatric hospitals.that use Joint Commission accreditation for deemed status purposes: The patient's goals include both short- and long-term goals.

Corrective Action:

Notes:

Moderate Sedation

45. Does the pre-procedure assessment include baseline level of consciousness (LOC) documentation? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.01.03 EP 1 - Before operative or other high-risk procedures are initiated or before anesthesia is administered, the hospital conducts a preanesthesia patient assessment.

Corrective Action:

Notes:

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46. Is the provider pre-sedation assessment completed and documented immediately prior to administration of sedation? Note: Ensure the assessment includes all required documentation as defined in policy (history, focused physical exam, airway assessment including Mallampati score, provisional diagnosis, and ASA physical status score) and does NOT contain arrows through boxes or slash marks through required fields. ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.13.01.03 EP 1 - Before operative or other high-risk procedures are initiated or before anesthesia is administered, the hospital conducts a preanesthesia patient assessment.

Corrective Action:

Notes:

47. Are vital signs and sedation assessment documented at 5–15 minute intervals after the administration of sedative or analgesic medications. NOTE: Minimum required documentation BP, heart rate, respiratory rate, O2 saturation, ETCO2, and sedation score. ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.02.01 EP 9 - The hospital defines, in writing, the scope and content of screening, assessment, and reassessment. Patient information is collected according to these requirements. Note 1: In defining the scope and content of the information it collects, the hospital may want to consider information that it can obtain, with the patient's consent, from the patient's family and the patient's other care providers, as well as information conveyed on any medical jewelry. Note 2: Assessment and reassessment information includes the patient's perception of the effectiveness of, and any side effects related to, their medication(s).

Corrective Action:

Notes:

48. Were post procedure vitals documented every 15-30 minutes until patient meets discharge criteria. NOTE: Discharge Criteria definitions in Instructions Below ☐ N/A

Instructions: Assess readiness for discharge using the Modified Aldrete tool.
The patient is discharged from the post-sedation or postanesthesia care area either by the physician or other licensed practitioner responsible for the patient's care according to the following discharge criteria:
Patients should be alert and oriented to their baseline status prior to discharge.
Cardiovascular function, airway patency, and protective airway reflexes are satisfactory at or above baseline assessment pre-procedure.
Vital signs should be stable, consistent with age specific parameters or near the patient's baseline.
Nausea controlled, able to tolerate fluids.
Mobility at pre-procedure baseline.

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NR.11.01.01 EP 2 - All licensed nurses who provide services in the hospital adhere to its policies and procedures. Note: This applies to all nursing staff providing services (that is, hospital employee, contract, lease, other agreement, or volunteer).

Corrective Action:

Notes:

49. Does the post-procedure / discharge assessment include modified Aldrete score? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

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PC.13.01.03 EP 2 - For hospitals that use Joint Commission accreditation for deemed status purposes: The hospital develops and implements policies and procedures for anesthesia that include the delineation of preanesthesia and postanesthesia responsibilities. The policies require the following for each patient: - A preanesthesia evaluation completed and documented by an individual qualified to administer anesthesia, as specified in 42 CFR 482.52(a), within 48 hours prior to surgery or a procedure requiring anesthesia services. - An intraoperative anesthesia record. - A postanesthesia evaluation completed and documented by an individual qualified to administer anesthesia, as specified in 42 CFR 482.52(a), no later than 48 hours after surgery or a procedure requiring anesthesia services. The postanesthesia evaluation for anesthesia recovery is completed in accordance with state law and hospital policies and procedures that have been approved by the medical staff and reflect current standards of anesthesia care.

Corrective Action:

Notes:

Titration

50. Was the titration started at the prescribed starting dose? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

MM.16.01.01 EP 1 - Drugs and biologicals are prepared and administered in accordance with federal and state laws, the orders of the licensed practitioner or practitioners responsible for the patient's care, and accepted standards of practice. For hospitals that use Joint Commission Accreditation for deemed status purposes: Drugs and biologicals may be prepared and administered as follows: - On the orders of other practitioners not specified under 42 CFR 482.12(c) only if such practitioners are acting in accordance with state law, including scope-of-practice laws, hospital policies, and medical staff bylaws, rules, and regulations. - On the orders contained within preprinted and electronic standing orders, order sets, and protocols for patient orders only if such orders meet the requirements of 42 CFR 482.24(c)(3).

Corrective Action:

Notes:

51. Was the medication(s) titrated and/or tapered according to the prescribed dose increments? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

MM.16.01.01 EP 1 - Drugs and biologicals are prepared and administered in accordance with federal and state laws, the orders of the licensed practitioner or practitioners responsible for the patient's care, and accepted standards of practice. For hospitals that use Joint Commission Accreditation for deemed status purposes: Drugs and biologicals may be prepared and administered as follows: - On the orders of other practitioners not specified under 42 CFR 482.12(c) only if such practitioners are acting in accordance with state law, including scope-of-practice laws, hospital policies, and medical staff bylaws, rules, and regulations. - On the orders contained within preprinted and electronic standing orders, order sets, and protocols for patient orders only if such orders meet the requirements of 42 CFR 482.24(c)(3).

Corrective Action:

Notes:

52. Was there a parameter assessment documented (i.e. RASS score, Blood pressure, Heart Rate, etc.) with every change in rate according to the order? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

MM.16.01.01 EP 1 - Drugs and biologicals are prepared and administered in accordance with federal and state laws, the orders of the licensed practitioner or practitioners responsible for the patient's care, and accepted standards of practice. For hospitals that use Joint Commission Accreditation for deemed status purposes: Drugs and biologicals may be prepared and administered as follows: - On the orders of other practitioners not specified under 42 CFR 482.12(c) only if such practitioners are acting in accordance with state law, including scope-of-practice laws, hospital policies, and medical staff bylaws, rules, and regulations. - On the orders contained within preprinted and electronic standing orders, order sets, and protocols for patient orders only if such orders meet the requirements of 42 CFR 482.24(c)(3).

Corrective Action:

Notes:

Informed consent

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53. Informed Consent was obtained prior to procedure by the provider and documented in the medical record? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.07.02.01 EP 1 - The hospital develops and implements a written policy on informed consent that describes the following: - Specific care, treatment, and services that require informed consent - Circumstances that would allow for exceptions to obtaining informed consent - Process used to obtain informed consent - Physicians or other licensed practitioners permitted to conduct the informed consent discussion in accordance with law and regulation - How informed consent is documented in the patient record Note: Documentation may be recorded in a form, in progress notes, or elsewhere in the record. - When a surrogate decision-maker may give informed consent

Corrective Action:

Notes:

54. Informed Consent Written Permit is documented in the medical record. ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

RC.12.01.01 EP 3 - The medical record contains any informed consent, when required by hospital policy or federal or state law or regulation. Note: The properly executed informed consent is placed in the patient's medical record prior to surgery, except in emergencies. A properly executed informed consent contains documentation of a patient's mutual understanding of and agreement for care, treatment, and services through written signature; electronic signature; or, when a patient is unable to provide a signature, documentation of the verbal agreement by the patient or surrogate decision-maker.

Corrective Action:

Notes:

55. Informed Consent Written Permit is completed in its entirety according to policy (legible, full procedure name (including sedation if required), no abbreviations, signed, dated, and timed) ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.07.02.01 EP 1 - The hospital develops and implements a written policy on informed consent that describes the following: - Specific care, treatment, and services that require informed consent - Circumstances that would allow for exceptions to obtaining informed consent - Process used to obtain informed consent - Physicians or other licensed practitioners permitted to conduct the informed consent discussion in accordance with law and regulation - How informed consent is documented in the patient record Note: Documentation may be recorded in a form, in progress notes, or elsewhere in the record. - When a surrogate decision-maker may give informed consent

Corrective Action:

Notes:

Immediate post operative note

56. Post op note is completed in the entirety, including the following: name of the primary surgeon and assistants, findings, technical procedures used, specimens removed and disposition of each specimen, post operative diagnosis, estimated blood loss (EBL), and authentication (sign, date, time). ☐ N/A

Instructions: May be N/A if the operative report is completed prior to transferring to a higher level of care.

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

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RC.12.01.03 EP 2 - An operative report is written or dictated immediately following surgery and signed by the surgeon. The report includes the following: - Name and hospital identification number of the patient - Date and times of the surgery - Name (s) of the surgeon(s) and assistants or other practitioners who performed surgical tasks (even when performing those tasks under supervision) and a description of the specific significant surgical tasks that were conducted by practitioners other than the primary surgeon/practitioner (significant surgical procedures include opening and closing, harvesting grafts, dissecting tissue, removing tissue, implanting devices, altering tissues) - Preoperative and postoperative diagnosis - Name of the specific surgical procedure(s) performed - Type of anesthesia administered - Complications, if any - Description of techniques, findings, and tissues removed or altered - Prosthetic devices, grafts, tissues, transplants, or devices implanted, if any - Any estimated blood loss Note 1: The exception to this requirement occurs when an operative or other high-risk procedure progress note is written immediately after the procedure, in which case the full report can be written or dictated within a time frame defined by the hospital. Note 2: If the physician or other licensed practitioner performing the operation or high-risk procedure accompanies the patient from the operating room to the next unit or area of care, the report can be written or dictated in the new unit or area of care.

Corrective Action:

Notes:

57. Immediate Post operative note is completed before the patient is transferred to the next level of care? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

RC.12.01.03 EP 2 - An operative report is written or dictated immediately following surgery and signed by the surgeon. The report includes the following: - Name and hospital identification number of the patient - Date and times of the surgery - Name (s) of the surgeon(s) and assistants or other practitioners who performed surgical tasks (even when performing those tasks under supervision) and a description of the specific significant surgical tasks that were conducted by practitioners other than the primary surgeon/practitioner (significant surgical procedures include opening and closing, harvesting grafts, dissecting tissue, removing tissue, implanting devices, altering tissues) - Preoperative and postoperative diagnosis - Name of the specific surgical procedure(s) performed - Type of anesthesia administered - Complications, if any - Description of techniques, findings, and tissues removed or altered - Prosthetic devices, grafts, tissues, transplants, or devices implanted, if any - Any estimated blood loss Note 1: The exception to this requirement occurs when an operative or other high-risk procedure progress note is written immediately after the procedure, in which case the full report can be written or dictated within a time frame defined by the hospital. Note 2: If the physician or other licensed practitioner performing the operation or high-risk procedure accompanies the patient from the operating room to the next unit or area of care, the report can be written or dictated in the new unit or area of care.

Corrective Action:

Notes:

Telemetry

58. Was the telemetry strip interpreted and documented every shift? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

RC.11.01.01 EP 4 - The hospital develops and implements policies and procedures for accurate, legible, complete, signed, dated, and timed medical record entries that are authenticated by the person responsible for providing or evaluating the service provided. The medical records are promptly completed, properly filed and retained, and readily accessible.

Corrective Action:

Notes:

59. Are all elements of documentation completed per policy? ☐ N/A

Instructions: Date and time
Patient name and room number
Heart rate
P.R. interval
QRS duration and QT segment
Rhythm interpretation including type of abnormal beats
Initials

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Compliant? ◯ Yes ◯ No	Num: _____ Den: _____

PC.11.02.01 EP 9 - The hospital defines, in writing, the scope and content of screening, assessment, and reassessment. Patient information is collected according to these requirements. Note 1: In defining the scope and content of the information it collects, the hospital may want to consider information that it can obtain, with the patient's consent, from the patient's family and the patient's other care providers, as well as information conveyed on any medical jewelry. Note 2: Assessment and reassessment information includes the patient's perception of the effectiveness of, and any side effects related to, their medication(s).

Corrective Action:

Notes:

Critical Results

60. RN notified the MD of the critical value within the hour ☐ N/A

Compliant? ◯ Yes ◯ No	Num: _____ Den: _____

NPG.01.02.01 EP 1 - The hospital develops and implements written procedures for managing the critical results of tests and diagnostic procedures that address the following: - The definition of critical results of tests and diagnostic procedures - By whom and to whom critical results of tests and diagnostic procedures are reported - The acceptable length of time between the availability and reporting of critical results of tests and diagnostic procedures

Corrective Action:

Notes:

61. MD did follow up or respond within the hour ☐ N/A

Compliant? ◯ Yes ◯ No	Num: _____ Den: _____

NPG.01.02.01 EP 1 - The hospital develops and implements written procedures for managing the critical results of tests and diagnostic procedures that address the following: - The definition of critical results of tests and diagnostic procedures - By whom and to whom critical results of tests and diagnostic procedures are reported - The acceptable length of time between the availability and reporting of critical results of tests and diagnostic procedures

Corrective Action:

Notes:

62. Communication of Critical Value is documented in the chart ☐ N/A

Compliant? ◯ Yes ◯ No	Num: _____ Den: _____

NPG.01.02.01 EP 1 - The hospital develops and implements written procedures for managing the critical results of tests and diagnostic procedures that address the following: - The definition of critical results of tests and diagnostic procedures - By whom and to whom critical results of tests and diagnostic procedures are reported - The acceptable length of time between the availability and reporting of critical results of tests and diagnostic procedures

Corrective Action:

Notes:

Admission Process / Screenings

63. Was an abuse/neglect screening conducted based on the established criteria of the hospital for screening? ☐ N/A

Compliant? ◯ Yes ◯ No	Num: _____ Den: _____

NPG.07.03.01 EP 1 - The hospital uses written criteria to identify those patients who may be victims of physical assault, sexual assault, sexual molestation, domestic abuse, elder or child abuse, neglect, and exploitation. Patients are evaluated upon entry into the hospital and on an ongoing basis. Note: Criteria can be based on age, sex, and circumstance.

Corrective Action:

Notes:

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64. Review the medical record. Have oral and written communication needs been identified for this patient, including the patient's preferred language for discussing healthcare? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.02.07 EP 1 - The hospital identifies the patient's oral and written communication needs, including the patient's preferred language for discussing health care. Note: Examples of communication needs include the need for personal devices such as hearing aids or glasses, language interpreters, communication boards, and translated or plain language materials.

Corrective Action:

Notes:

Falls

65. Adult inpatient assessed on admission for fall risk using the Morse Fall Risk Scale? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.11.02.01 EP 1 - The hospital implements fall risk reduction interventions based on the patient population, setting, and individual patient's assessed risks.

Corrective Action:

Notes:

66. Adult inpatient reassessed for fall risk at minimum twice daily (within 24 hours), or whenever the adult inpatient condition changes? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.11.02.01 EP 1 - The hospital implements fall risk reduction interventions based on the patient population, setting, and individual patient's assessed risks.

Corrective Action:

Notes:

67. Fall Risk Care Plan matches the risk factors identified in Morse Fall Risk Scale? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.03.01 EP 1 - The hospital develops, implements, and revises a written individualized plan of care based on the following: - Needs identified by the patient's assessment, reassessment, and results of diagnostic testing - The patient's goals and the time frames, settings, and services required to meet those goals Note 1: Nursing staff develops and keeps current a nursing plan of care, which may be a part of an interdisciplinary plan of care, for each patient. Note 2: The hospital evaluates the patient's progress and revises the plan of care based on the patient's progress. Note 3: For psychiatric hospitals that use Joint Commission accreditation for deemed status purposes: The patient's goals include both short- and long-term goals.

Corrective Action:

Notes:

68. If the patient scored as high risk is the patient wearing a yellow gown or a yellow wristband? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.11.02.01 EP 1 - The hospital implements fall risk reduction interventions based on the patient population, setting, and individual patient's assessed risks.

Corrective Action:

Notes:

Nutrition

69. Meal intake was documented for the last three meals in the past 24 hours? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

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PC.12.01.09 EP 1 - The nutritional needs of the individual patient are met in accordance with clinical practice guidelines and recognized dietary practices. Note: Diet menus meet the needs of the patients.

Corrective Action:

Notes:

I&O

70. Intake and Output was documented at a minimum of every 8 hours? ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

PC.11.02.01 EP 9 - The hospital defines, in writing, the scope and content of screening, assessment, and reassessment. Patient information is collected according to these requirements. Note 1: In defining the scope and content of the information it collects, the hospital may want to consider information that it can obtain, with the patient's consent, from the patient's family and the patient's other care providers, as well as information conveyed on any medical jewelry. Note 2: Assessment and reassessment information includes the patient's perception of the effectiveness of, and any side effects related to, their medication(s).

Corrective Action:

Notes:

Workplace Violence

71. Has the Violence Aggression Assessment Checklist (VAAC) be documented in EPIC for the reviewed encounter and appropriate interventions documented for a high risk screening ☐ N/A

Compliant? ◯ Yes ◯ No | Num: _____ Den: _____

NPG.02.04.01 EP 3 - The hospital conducts an annual worksite analysis related to its workplace violence prevention program. The hospital takes actions to mitigate or resolve the workplace violence safety and security risks based on findings from the analysis. Note: A worksite analysis includes a proactive analysis of the worksite, an investigation of the hospital's workplace violence incidents, and an analysis of how the program's policies and procedures, training, education, and environmental design reflect best practices and conform to applicable laws and regulations.

Corrective Action:

Notes:

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